

Important information. Please read carefully.

Sofilex[®]

For Oral Suspension

Cephalexin Broad Spectrum Antibiotic

Composition

Each 5ml when reconstituted contains Cephalexin 125mg. Sodium Benzoate 0.01g as preservative.

Pharmacodynamics

Cephalexin is a beta-lactam antibiotic. It is bactericidal and acts by inhibiting synthesis of the bacterial cell wall. It has good activity against a wide spectrum of Gram-positive bacteria and it is most active against Gram-positive cocci. It has moderate activity against some Gram-negative bacilli. Sensitive Gram-positive cocci include both penicillinase and non-penicillinase-producing staphylococci.

Pharmacokinetics

Cephalexin is almost completely absorbed from the gastro-intestinal tract. If cephalexin is taken with food, absorption may be delayed, but the total amount absorbed is not appreciably altered. Up to 15% of a dose is bound to plasma proteins. The plasma half-life is about 1 hour; it increases with reduced renal function. Cephalexin is widely distributed in the body but does not enter the cerebrospinal fluid in significant quantities. It crosses the placenta and small quantities are found in the milk of nursing mothers. Cephalexin is not metabolised. About 80% or more of a dose is excreted unchanged in the urine, some may be excreted in the bile.

Indications

Cephalexin is administered by mouth for the treatment of susceptible infections including those of the respiratory, urinary tracts and of the skin. It is also indicated in upper respiratory infections including pharyngitis and tonsillitis (caused by *Streptococcus pyogenes*) and otitis media (caused by *Str. pneumoniae*, *H. Influenzae*, *Staphylococci* and *S. pyogenes*.)

Dosage and Administration

For oral administration only.

To Reconstitute: Shake to loosen powder.

Add water of at least 30ml and shake vigorously to disperse powder. Add water to the 60ml mark and shake well.

Child under 1 year: 5ml every 12 hours

Child 1-5 years: 5ml every 8 hours

Child 6-12 years: 10ml every 8 hours

Contraindications

Allergy to Cephalosporins.

Precautions

Cephalexin should be given with caution to patients with renal impairment; a dosage reduction may be necessary. Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Sofilex, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Sofilex must be discontinued immediately and appropriate alternative therapy instituted.

Drug Interactions

Cephalexin may interfere with method of measuring creatinine concentration, the direct Coombs' test and urine glucose test using copper-reduction reactions.

There may also be enhanced nephrotoxicity with concomitant use of nephrotoxic drug such as aminoglycoside and loop diuretic like frusemide. Probenecid delays urinary excretion of cephalexin. There may be antagonism between cephalexin and bacteriostatic antibacterial agents.

Incompatibilities

None known.

Usage during Pregnancy

As with all other drugs, it should be administered with caution during early months of pregnancy.

Side Effects

The most common adverse effects of Cephalexin and other oral Cephalosporins are generally gastrointestinal disturbances (including nausea, vomiting and diarrhoea) and hypersensitivity reactions (including skin rashes, urticaria, eosinophilia, fever, reactions resembling serum sickness and anaphylaxis). Pseudomembranous colitis has been reported. Neutropenia, thrombocytopenia and agranulocytosis have occasionally been reported. Allergic reactions have been observed in the form of rash, urticaria, angioedema and rarely erythema multiforme, Stevens-Johnson syndrome and toxic epidermal necrolysis. These reactions subsided upon discontinuation of the drug, although in some cases, supportive therapy may be necessary.

Overdosage and Treatment

Symptoms: Symptoms of oral overdose may include nausea, vomiting, epigastric distress, diarrhoea and hematuria.

Treatment: Unless 5 to 10 times the normal dose of Cephalexin has been ingested, gastrointestinal decontamination should not be necessary. Protect the patient's airway and support ventilation and perfusion. To decrease gastrointestinal absorption of Cephalexin, charcoal can be considered instead of or in addition to gastric emptying. Cephalexin is removed by haemodialysis and peritoneal dialysis.

Shelf-life

The expiry date is indicated on the packaging.

Presentation

Before reconstitution: White colour powder in 60ml plastic containers

After reconstitution: Off-white colour, blackcurrant flavour suspension in 60ml plastic containers.

Reconstitution

Shake the bottle to loosen powder. Add water of at least 30ml. Shake to disperse powder. Let the suspension stand for 10 to 20 seconds and add a further amount of water to the respective 60 ml mark. Shake well.

Use within 7 days after reconstitution.

Storage

Keep container well closed. Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN

JAUHI DARI KANAK-KANAK

For further information, please consult your pharmacist or physician.

Revision Date : 19-Sep-2019



Xepa^{SP}

Manufacturer and Product Registration Holder

Xepa-Soul Pattinson (Malaysia) Sdn Bhd

1-5 Cheng Industrial Estate, 75250 Melaka, Malaysia.